INTERNAL WORKING DOCUMENT

# Source Quality Assessment

India Healthcare M&A Sector Scan: review of the Source Trace Report

Reviewed document: India_Healthcare_Source_Trace_Report.docx (31 October 2026) | Audience: engagement team

## 1. Bottom line

**Overall recommendation: USE WITH CAUTION.**

The report picks the right kinds of source. Nearly all are primary government or regulatory bodies, and each claim maps to a real diligence question. It is a sound source map and checklist.

It is not yet an evidence base we can put in front of a client. It cites document types and not documents: no editions, dates, order numbers, URLs or access dates. One citation is a placeholder, and several confidence labels are higher than the underlying sources justify. Of the 14 claims, I would downgrade 7 (Claims 3, 4, 5, 9, 10, 11 and 13), leave the other 7 unchanged, and upgrade none.

The report's own statement that every claim was matched against sources "without assumptions" cannot be tested from the document, because no source content is reproduced.

## 2. Scorecard across twelve dimensions

Ratings are my own, based only on what the report document contains. Where the report is silent, I have rated the dimension "not assessable" and not assumed good practice.

| Dimension | Rating | Basis |
|---|---|---|
| **Authority** | **HIGH** | Sources named are statutory bodies and regulators (MoHFW, MoSPI, DGCI&S, CCI, SEBI, CDSCO, NPPA, NHA, IRDAI, MeitY). IVCA is the one industry body and the report correctly labels it secondary. |
| **Relevance** | **HIGH** | Each of the 14 claims maps to an M&A diligence question: market sizing, deal terms, licensing, pricing, quality, digital, AI, data privacy, revenue quality, regulatory outlook. |
| **Recency** | **MEDIUM** | The report is dated 31 Oct 2026 but gives no publication date or reporting period for any source. Recency cannot be confirmed. The report itself notes the NHA publication lag (Claim 1). |
| **Evidence** | **MEDIUM** | Source types are primary in nature (registers, orders, filings), but the report names them and does not extract or attach evidence. Nothing in it lets a reader check a figure. |
| **Independence** | **MEDIUM-HIGH** | Low commercial bias. However, government dashboards (ABDM, PM-JAY) are self-reported by the bodies that run them, so authority is not the same as independence. The report does not state a policy on promotional or company-sourced material. |
| **Traceability** | **MEDIUM-LOW** | Citations stop at document type ("NHA Reports", "NPPA ceiling price Orders"). No edition, year, order or gazette number, URL or access date. Claim 10 contains a literal placeholder: "Regulation (EU) 2023/XXX". |
| **Methodology** | **NOT ASSESSABLE** | The report describes no methodology for any source, and none of its source methodologies is examined. Any rating here would be inferred from general knowledge of NHA and NSS, not from the document. |
| **Definitions** | **MEDIUM** | No market boundary is defined (what counts as "healthcare sector", which segments, which fiscal years). Source classifications such as HS codes are implied but not specified. |
| **Comparability** | **MEDIUM** | Government series are generally comparable over time, but the report does not say which years or series are being compared, so comparability is plausible and unverified. |

| Dimension | Rating | Basis |
|---|---|---|
| **Forecast quality** | **NOT ASSESSABLE** | The report contains no forecasts. Claim 14 covers draft regulation only. The conclusion mentions "market forecasts" that do not appear in the document. |
| **AI / data governance relevance** | **MEDIUM** | Claims 9, 10, 11 and 14 cover the right instruments, but Claim 9 overgeneralises (only AI software qualifying as a medical device needs a CDSCO licence; ICMR and IndiaAI material is advisory) and Claim 11 treats general data law as healthcare-specific. |
| **Regulatory reliability** | **MEDIUM-HIGH** | Primary legal instruments are the right basis and drafts are separated from binding rules. Reliability is reduced by the placeholder EU citation and by unconfirmed commencement dates for the DPDP Rules. |

## 3. Claim-by-claim review

"Report rating" is the confidence level the report assigned. "Revised" is my view of how well the cited sources, as documented, support the claim.

| No. | Report rating | Revised | Finding and action |
|---|---|---|---|
| **1** | High | **HIGH** | NHA and NSS are the right sources. State the NHA edition and fiscal year, since the report itself notes a publication lag. |
| **2** | High | **HIGH** | DGCI&S data is definitive for trade flows. HS-level extraction needs access, and company-level exposure is out of scope. |
| **3** | High | **MEDIUM-HIGH** | CCI orders only cover deals above notification thresholds, so they are not a complete measure of consolidation. IVCA is secondary and cited by an outdated name. |
| **4** | High | **MEDIUM-HIGH** | Filings give deal terms; "multiples" are usually derived by the analyst. Most healthcare deals are private and invisible here. Source organisation lists SEBI only, although exchange filings are company-originated. |
| **5** | High | **MEDIUM-HIGH** | The licence register confirms status. It does not answer transferability on a change of control, which is a legal question needing counsel and the specific licence conditions. |
| **6** | Medium-high | **MEDIUM-HIGH** | Fair rating (accreditation is a proxy). "GMC registrations" is ambiguous; name the State Medical Councils or the National Medical Register. |
| **7** | High | **HIGH** | Ceiling prices and package rates are binding and published. Specify the order or notification date and the package version. |
| **8** | High | **HIGH** | Dashboards give counts. They do not show quality or utilisation depth, which the report already notes. Record the snapshot date. |
| **9** | Medium-high | **MEDIUM** | Overgeneralised: licensing applies only to AI software that qualifies as a medical device. ICMR and IndiaAI frameworks are advisory, not binding. |
| **10** | High | **MEDIUM-HIGH** | Right idea, defective citation ("2023/XXX"). The EU AI Act is Regulation (EU) 2024/1689. FDA guidance should be cited by title and date. |
| **11** | High | **MEDIUM-HIGH** | The DPDP Act has no healthcare-specific provisions. Confirm the commencement schedule of the 2025 Rules, since I believe key obligations phase in over 12 to 18 months from notification. |
| **12** | Medium-high | **MEDIUM-HIGH** | NSQ alerts and pharmacovigilance reports are valid early signals. PLFS has limited granularity on healthcare occupations, so workforce evidence is thin. |
| **13** | High | **MEDIUM** | Granular claim rejection rates and payment delay data are not clearly public at PM-JAY or IRDAI level. Audited receivables from MCA filings are the more reliable part of this claim. |
| **14** | Medium | **MEDIUM** | Appropriately rated. Drafts are signals only. |

Net effect: the report's mix of 10 High, 3 Medium-high and 1 Medium becomes roughly 4 High, 7 Medium-high and 3 Medium on my reading.

## 4. Fitness for use

| Use | Verdict | Condition |
|---|---|---|
| **Background context** | **HIGH** | Suitable now for framing the sector and the regulatory landscape. |
| **Factual statements (market size, regulatory status, deal history)** | **MEDIUM-HIGH** | Only once each statement carries a dated, specific citation. Public deals only. |
| **Risk assessment** | **MEDIUM-HIGH** | Good for identifying what to test. Quality and operational signals are proxies and need site validation. |
| **Forecasts and projections** | **LOW** | The report contains none, and draft regulation cannot support firm projections. |
| **Investment conclusions** | **MEDIUM** | Only together with deal-specific commercial, legal and operational diligence. |

## 5. Recommended actions

1. **Close the citation gap.** For every claim, add title, issuing body, date or fiscal year, order or gazette number, URL and access date. Do this before any content is reused in a client document.
2. **Fix the placeholder in Claim 10.** Replace "Regulation (EU) 2023/XXX" with the correct instrument (I believe Regulation (EU) 2024/1689) and cite FDA guidance by title and date.
3. **Verify DPDP commencement.** Pull the DPDP Rules 2025 from the Gazette and confirm which obligations are in force today. Narrow Claim 11 to say what the Act actually covers for health data.
4. **Tighten Claim 9.** Rewrite Claim 9 to limit the licence requirement to AI software qualifying as a medical device, and mark ICMR and IndiaAI material as advisory.
5. **Re-base Claims 4 and 13.** For Claims 4 and 13, identify what is actually public (offer documents, MCA21 financials) and drop or qualify rejection-rate and multiple statements that cannot be sourced.
6. **Define scope.** Add a short statement of market boundaries, fiscal years and units, so figures are comparable across sources.
7. **Plan target-specific verification.** For the target, confirm licence transferability with counsel, run site-level checks beyond NABH status, and review private-deal data through diligence requests.

## 6. Limits of this review

This review is based on the report document alone. I did not open or retrieve any of the underlying sources. Statements about the EU AI Act citation and DPDP Rules timing come from my own knowledge and are flagged for verification against the primary texts before anyone relies on them.